

Erectile Dysfunction

Patient Group Direction, version 003, issued 9 September 2026. Sildenafil and tadalafil, adult males aged 18 and over.

The two things that changed, and why

- **THE NITRATE EXCLUSION NOW NAMES NICORANDIL AND AMYL NITRITE, IN BOTH ARMS.** Version 001 named nitric oxide donors and amyl nitrite in the sildenafil arm only. The tadalafil arm said just "organic nitrates in any form", so poppers were invisible in the arm most likely to be requested by the men most likely to use them. Nicorandil appeared in neither arm at all.
- **THE CARDIOVASCULAR FITNESS CRITERION NOW HAS CONTENT.** Appendix 1 sets out the question to ask and what to do with the answer.
- **The alpha-blocker starting dose and the over-65 dose reduction, which the sildenafil arm had and the tadalafil arm had lost, now appear in both.**
- **These are not paperwork changes. A PDE5 inhibitor with a nitric oxide donor can kill, and the patient will not volunteer that he uses poppers unless he is asked directly.**

Sildenafil

Patient Group Direction for the supply of sildenafil 25mg, 50mg and 100mg film-coated tablets for the treatment of erectile dysfunction in adult males aged 18 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on erectile dysfunction.
- ALL USERS MUST BE ABLE TO RECOGNISE EVERY NITRIC OXIDE DONOR BY NAME, not only the medicines with "nitrate" in the title. That includes nicorandil, which is an angina medicine, and amyl nitrite or "poppers", which is bought recreationally and will not appear on any medication list.
- All users must be able to apply the cardiovascular fitness assessment in Appendix 1 and to record its result.
- All users must understand that erectile dysfunction can be the first sign of cardiovascular disease or of undiagnosed diabetes, and must be willing to refer rather than treat where the history suggests it.
- All users must previously have supplied medicines under a PGD.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of erectile dysfunction in adult males aged 18 years and over, being the inability to achieve or maintain a penile erection sufficient for satisfactory sexual performance.
Inclusion criteria	• Adult male aged 18 years and over. • Clinical presentation consistent with erectile dysfunction. • Cardiovascular fitness assessment in Appendix 1 completed, passed, and recorded. • Blood pressure measured today and within the acceptable range. • Nitrate and nitric oxide donor questions asked, including the direct question about poppers, and all answers negative. • Valid informed consent obtained. • No exclusion criterion present.
Nitrates and nitric oxide donors	ABSOLUTE EXCLUSION IN BOTH ARMS. The combination causes profound, prolonged and potentially fatal hypotension. Ask about every item on this list by name. • Glyceryl trinitrate, in any form: tablets, spray, patches, ointment, or an infusion. Including a GTN spray carried "just in case" and used rarely. • Isosorbide mononitrate or isosorbide dinitrate. • NICORANDIL. This is a potassium channel activator with a nitrate moiety and it is a nitric oxide donor. It is prescribed for angina and does not have "nitrate" in its name, which is why it gets missed. It appeared nowhere in version 001. • AMYL NITRITE, butyl nitrite, isobutyl nitrite or alkyl nitrites, commonly sold as "poppers". These are bought recreationally, are not prescribed, and will not appear on any medication list. YOU MUST ASK DIRECTLY. • Sodium nitroprusside. • Riociguat, and any other soluble guanylate cyclase stimulator. How to ask about poppers, so that the answer is useful: "Some men use poppers, amyl nitrite, at the same time as these tablets. Taken together they can drop your blood pressure to a dangerous level. Do you ever use them, or think you might?" Ask it without judgement and record the answer.

	Where the patient uses poppers and is unwilling to stop, do not supply. Explain why plainly: it is the combination that is dangerous, not either one alone.
Cardiovascular fitness	Apply the following and record the result. THE FUNCTIONAL TEST. Ask: "Can you walk a mile on the flat in about 20 minutes, or climb two flights of stairs briskly, without chest pain and without having to stop for breath?" • YES, comfortably: proceed. Record the answer given. • NO, or the patient does not know because he never exerts himself that much: DO NOT SUPPLY. Refer to the GP for cardiovascular assessment. Sexual activity carries a similar cardiac workload. • Chest pain, breathlessness or palpitations on exertion OR during previous sexual activity: DO NOT SUPPLY. Refer. Also record, at every supply: • Blood pressure measured today. Do not accept a figure from memory. • Whether the patient has known cardiovascular disease, and if so what. • Date of the last cardiovascular review, where known. Erectile dysfunction commonly precedes a cardiovascular event by two to five years and is frequently the first presentation of undiagnosed diabetes. A man attending for this is an opportunity, not just a customer. Where he has not had a recent check, recommend one and record that you did.
Exclusion criteria	Refer, do not supply, where any of the following applies. • Known hypersensitivity to sildenafil or to any excipient. • Under 18 years of age. Female. • ANY nitrate or nitric oxide donor, including nicorandil and amyl nitrite. See the row above. • Riociguat or any other soluble guanylate cyclase stimulator. • Recent stroke or myocardial infarction, within the last 6 months. • Hypotension, blood pressure below 90/50 mmHg, measured today. • Uncontrolled hypertension, blood pressure above 170/100 mmHg, measured today. • Fails the cardiovascular fitness assessment in Appendix 1, or it has not been done. • Unstable angina, angina occurring during sexual activity, severe heart failure, or uncontrolled arrhythmia. • Known hereditary degenerative retinal disorder, for example retinitis pigmentosa. • Previous episode of non-arteritic anterior ischaemic optic neuropathy. • Previous priapism, or an erection lasting more than 4 hours on any previous PDE5 inhibitor. • Already taking any other PDE5 inhibitor, including one obtained online or from another supplier. Do not add a second. • Erectile dysfunction of sudden onset following trauma, surgery or a new medicine, or accompanied by penile pain or deformity. Refer for a diagnosis rather than treating the symptom. • Valid informed consent not obtained. • Severe hepatic impairment.
Cautions	Anatomical deformation of the penis: angulation, cavernosal fibrosis or Peyronie's disease. Conditions predisposing to priapism: sickle cell anaemia, multiple myeloma, leukaemia. Active peptic ulceration or a bleeding disorder.

	Advise the patient that sudden visual loss may indicate NAION, and sudden hearing loss has also been reported. Stop the medicine and seek urgent medical attention for either. These medicines do not protect against sexually transmitted infections. Say so, and offer testing where appropriate. Sexual stimulation is required. The tablet does not produce an erection by itself. Failure to explain this is the commonest reason a patient reports the medicine "does not work". ALPHA-BLOCKERS. The patient must be stable on his alpha-blocker before starting. START AT 25mg. Do not start at 50mg. AGED OVER 65. Start at 25mg. CYP3A4 inhibitors, for example erythromycin, clarithromycin, ketoconazole, itraconazole or ritonavir. Start at 25mg. Mild to moderate hepatic impairment. Start at 25mg. Severe renal impairment, eGFR below 30 mL/min. Start at 25mg. Avoid grapefruit juice, which increases exposure.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where the exclusion is cardiovascular, say so plainly and recommend a GP assessment: erectile dysfunction is frequently the first sign of cardiovascular disease, and a referral here is worth more to the patient than a tablet. Where the exclusion is poppers, explain that it is the combination that is dangerous. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Sildenafil 25mg, 50mg and 100mg film-coated tablets.
Legal category	POM.
Dose and frequency	- Starting dose 50mg as needed, about 1 hour before sexual activity. START AT 25mg where the patient is over 65, on an alpha-blocker, on a CYP3A4 inhibitor, or has hepatic or severe renal impairment. - May be increased to a maximum of 100mg, or decreased to 25mg, on efficacy and tolerability. MAXIMUM ONE DOSE IN ANY 24 HOURS. - Efficacy may be reduced after a high-fat meal. - Can be taken from 30 minutes to 4 hours before sexual activity. Sexual stimulation is required.
Quantity to be supplied	Up to 8 tablets per supply, being about one month at twice-weekly use. Decide the quantity on individual need and record the decision. Do not supply more than 8.
Maximum or minimum treatment period	No maximum treatment period. Review effectiveness, blood pressure and cardiovascular fitness at least annually, and reassess where there has been no improvement after 6 to 8 attempts at the maximum tolerated dose.
Route and method	Oral.
Storage	Store below 30 degrees Celsius in the original packaging.
Adverse effects	Very common: headache. Common: flushing, dyspepsia, nasal congestion, dizziness, visual disturbance including a colour tinge, increased brightness or blurred vision, nausea. Uncommon: vomiting, rash, epistaxis, palpitations, tachycardia, drowsiness, eye pain, tinnitus. Rare but serious: priapism, non-arteritic anterior ischaemic optic neuropathy, sudden sensorineural hearing loss.

Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • THE ANSWER TO THE NITRATE AND NITRIC OXIDE DONOR QUESTIONS, including the direct question about poppers. • THE CARDIOVASCULAR FITNESS ANSWER, in the patient's terms, and the blood pressure measured today. • Whether the patient is on an alpha-blocker, and if so which one and whether he is stable on it. • Age, and where over 65, the starting dose chosen. • Any CYP3A4 inhibitor, and any hepatic or renal impairment, and the starting dose chosen as a result. • Name of the healthcare practitioner and registration number. • Name, brand, form, strength, dose, route, quantity and date of supply. • That the patient was advised these medicines do not protect against sexually transmitted infections. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.
Disposal	Advise the patient to return unused tablets to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Take about an hour before sex. It can work from 30 minutes up to about 4 hours. • You still need to be aroused. The tablet does not do it on its own. • One tablet in 24 hours. No more. • A heavy, fatty meal can make it work less well. • Avoid grapefruit juice. • NEVER take this with poppers, or with any nitrate medicine such as a GTN spray or nicorandil. Together they can drop your blood pressure to a dangerous level. • This does not protect you against sexually transmitted infections.
Follow-up and urgent symptoms	Seek immediate medical attention for: • An erection lasting more than 4 hours. Go to A&E. This can cause permanent damage. • Sudden loss of vision in one or both eyes. Stop the tablets. • Sudden loss or reduction of hearing. • Chest pain during or after sex. Do not take a GTN spray for it; tell the paramedics you have taken this medicine. If you are not under regular GP review, arrange a cardiovascular and diabetes check. Erection problems are often the first sign of something else.

Tadalafil

Patient Group Direction for the supply of tadalafil 2.5mg, 5mg, 10mg and 20mg film-coated tablets for the treatment of erectile dysfunction in adult males aged 18 years and over, on demand or once daily.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on erectile dysfunction.
- ALL USERS MUST BE ABLE TO RECOGNISE EVERY NITRIC OXIDE DONOR BY NAME, not only the medicines with "nitrate" in the title. That includes nicorandil, which is an angina medicine, and amyl nitrite or "poppers", which is bought recreationally and will not appear on any medication list.
- All users must be able to apply the cardiovascular fitness assessment in Appendix 1 and to record its result.
- All users must understand that erectile dysfunction can be the first sign of cardiovascular disease or of undiagnosed diabetes, and must be willing to refer rather than treat where the history suggests it.
- All users must previously have supplied medicines under a PGD.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of erectile dysfunction in adult males aged 18 years and over. Available as on-demand dosing, or once-daily dosing for men who anticipate frequent sexual activity, at least twice per week.
Inclusion criteria	• Adult male aged 18 years and over. • Clinical presentation consistent with erectile dysfunction. • Cardiovascular fitness assessment in Appendix 1 completed, passed, and recorded. • Blood pressure measured today and within the acceptable range. • Nitrate and nitric oxide donor questions asked, including the direct question about poppers, and all answers negative. • Valid informed consent obtained. • No exclusion criterion present.
Nitrates and nitric oxide donors	ABSOLUTE EXCLUSION IN BOTH ARMS. The combination causes profound, prolonged and potentially fatal hypotension. Ask about every item on this list by name. • Glyceryl trinitrate, in any form: tablets, spray, patches, ointment, or an infusion. Including a GTN spray carried "just in case" and used rarely. • Isosorbide mononitrate or isosorbide dinitrate. • NICORANDIL. This is a potassium channel activator with a nitrate moiety and it is a nitric oxide donor. It is prescribed for angina and does not have "nitrate" in its name, which is why it gets missed. It appeared nowhere in version 001. • AMYL NITRITE, butyl nitrite, isobutyl nitrite or alkyl nitrites, commonly sold as "poppers". These are bought recreationally, are not prescribed, and will not appear on any medication list. YOU MUST ASK DIRECTLY. • Sodium nitroprusside. • Riociguat, and any other soluble guanylate cyclase stimulator. How to ask about poppers, so that the answer is useful: "Some men use poppers, amyl nitrite, at the same time as these tablets. Taken together they can drop your blood pressure to a dangerous level. Do you ever use them, or think you might?" Ask it without judgement and record the answer.

	Where the patient uses poppers and is unwilling to stop, do not supply. Explain why plainly: it is the combination that is dangerous, not either one alone.
Cardiovascular fitness	Apply the following and record the result. THE FUNCTIONAL TEST. Ask: "Can you walk a mile on the flat in about 20 minutes, or climb two flights of stairs briskly, without chest pain and without having to stop for breath?" • YES, comfortably: proceed. Record the answer given. • NO, or the patient does not know because he never exerts himself that much: DO NOT SUPPLY. Refer to the GP for cardiovascular assessment. Sexual activity carries a similar cardiac workload. • Chest pain, breathlessness or palpitations on exertion OR during previous sexual activity: DO NOT SUPPLY. Refer. Also record, at every supply: • Blood pressure measured today. Do not accept a figure from memory. • Whether the patient has known cardiovascular disease, and if so what. • Date of the last cardiovascular review, where known. Erectile dysfunction commonly precedes a cardiovascular event by two to five years and is frequently the first presentation of undiagnosed diabetes. A man attending for this is an opportunity, not just a customer. Where he has not had a recent check, recommend one and record that you did.
Exclusion criteria	Refer, do not supply, where any of the following applies. • Known hypersensitivity to tadalafil or to any excipient. • Under 18 years of age. Female. • ANY nitrate or nitric oxide donor, including nicorandil and amyl nitrite. See the row above. • Riociguat or any other soluble guanylate cyclase stimulator. • Recent stroke or myocardial infarction, within the last 6 months. • Hypotension, blood pressure below 90/50 mmHg, measured today. • Uncontrolled hypertension, blood pressure above 170/100 mmHg, measured today. • Fails the cardiovascular fitness assessment in Appendix 1, or it has not been done. • Unstable angina, angina occurring during sexual activity, severe heart failure, or uncontrolled arrhythmia. • Known hereditary degenerative retinal disorder, for example retinitis pigmentosa. • Previous episode of non-arteritic anterior ischaemic optic neuropathy. • Previous priapism, or an erection lasting more than 4 hours on any previous PDE5 inhibitor. • Already taking any other PDE5 inhibitor, including one obtained online or from another supplier. Do not add a second. • Erectile dysfunction of sudden onset following trauma, surgery or a new medicine, or accompanied by penile pain or deformity. Refer for a diagnosis rather than treating the symptom. • Valid informed consent not obtained. • Severe hepatic impairment, Child-Pugh class C. • Severe renal impairment, creatinine clearance below 30 mL/min, for once-daily dosing.
Cautions	Anatomical deformation of the penis: angulation, cavernosal fibrosis or Peyronie's disease. Conditions predisposing to priapism: sickle cell anaemia, multiple myeloma,

	leukaemia. Active peptic ulceration or a bleeding disorder. Advise the patient that sudden visual loss may indicate NAION, and sudden hearing loss has also been reported. Stop the medicine and seek urgent medical attention for either. These medicines do not protect against sexually transmitted infections. Say so, and offer testing where appropriate. Sexual stimulation is required. The tablet does not produce an erection by itself. Failure to explain this is the commonest reason a patient reports the medicine "does not work". PROLONGED HALF-LIFE, about 17.5 hours. Effects and adverse effects last considerably longer than with sildenafil, and so does the interaction with any nitrate. A patient who takes tadalafil on Friday is still at risk on Sunday. Say so. ALPHA-BLOCKERS. The patient must be stable on his alpha-blocker before starting. Start on-demand dosing at 10mg and do not exceed it until tolerance is established. For once-daily dosing, start at 2.5mg. AGED OVER 65. Start on-demand dosing at 10mg rather than escalating, and once-daily dosing at 2.5mg. Potent CYP3A4 inhibitors, for example ketoconazole or ritonavir. On-demand dose must not exceed 10mg in any 72 hour period. Mild to moderate hepatic impairment, Child-Pugh A or B. On-demand dose must not exceed 10mg. Moderate renal impairment, creatinine clearance 30 to 50 mL/min. For once-daily dosing start at 2.5mg.
Actions if the patient is excluded or declines	As for the sildenafil arm.

The medicine

Name, form and strength	Tadalafil 2.5mg, 5mg, 10mg and 20mg film-coated tablets.
Legal category	POM.
Dose and frequency	● ON DEMAND: starting dose 10mg, at least 30 minutes before sexual activity. May be increased to 20mg or decreased to 5mg on efficacy and tolerability. MAXIMUM ONE DOSE IN ANY 24 HOURS. ● ONCE DAILY: 2.5mg once daily at about the same time each day, increased to 5mg on tolerability. For men anticipating sexual activity at least twice per week. ● Do not use both regimens. Choose one and record which. ● Effective for up to 36 hours. Absorption is not affected by food. ● With a potent CYP3A4 inhibitor, on-demand use must not exceed 10mg in 72 hours. ● Sexual stimulation is required.
Quantity to be supplied	On demand: up to 8 tablets per supply. Once daily: up to 28 tablets, being one month. Decide the quantity on individual need and record the decision.
Maximum or minimum treatment period	No maximum treatment period. Review effectiveness, blood pressure and cardiovascular fitness at least annually. For once-daily dosing, reassess periodically whether continued daily use remains appropriate.
Route and method	Oral.
Storage	Store below 30 degrees Celsius in the original packaging. Protect from moisture.
Adverse effects	Very common: headache. Common: back pain, myalgia, pain in extremity,

	flushing, nasal congestion, dyspepsia, gastro-oesophageal reflux. Uncommon: dizziness, abdominal pain, hyperhidrosis, rash, tachycardia, palpitations. Rare but serious: priapism, non-arteritic anterior ischaemic optic neuropathy, sudden sensorineural hearing loss. Because of the prolonged half-life, adverse effects may persist longer than with sildenafil.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • THE ANSWER TO THE NITRATE AND NITRIC OXIDE DONOR QUESTIONS, including the direct question about poppers. • THE CARDIOVASCULAR FITNESS ANSWER, in the patient's terms, and the blood pressure measured today. • Whether the patient is on an alpha-blocker, and if so which one and whether he is stable on it. • Age, and where over 65, the starting dose chosen. • Any CYP3A4 inhibitor, and any hepatic or renal impairment, and the starting dose chosen as a result. • Name of the healthcare practitioner and registration number. • Name, brand, form, strength, dose, route, quantity and date of supply. • That the patient was advised these medicines do not protect against sexually transmitted infections. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.
Disposal	Advise the patient to return unused tablets to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• On demand: take at least 30 minutes before sex. It can still work up to 36 hours later. • Once daily: take at the same time each day, whether or not you expect to have sex. • You still need to be aroused. The tablet does not do it on its own. • One dose in 24 hours. No more. • NEVER take this with poppers, or with any nitrate medicine such as a GTN spray or nicorandil. Because this medicine stays in your system for up to two days, that warning applies for two days after your last dose, not just on the day. • This does not protect you against sexually transmitted infections.
Follow-up and urgent symptoms	Seek immediate medical attention for: • An erection lasting more than 4 hours. Go to A&E. • Sudden loss of vision in one or both eyes. Stop the tablets. • Sudden loss or reduction of hearing. • Chest pain during or after sex. Do not take a GTN spray for it; tell the paramedics you have taken this medicine, and that it lasts up to 36 hours. If you are not under regular GP review, arrange a cardiovascular and diabetes check.

Appendix 1: Cardiovascular fitness assessment

Complete for every patient, at every supply, and record the answers. Sexual activity carries a cardiac workload comparable to brisk walking or climbing two flights of stairs.

1. The functional question	"Can you walk a mile on the flat in about 20 minutes, or climb two flights of stairs briskly, without chest pain and without having to stop for breath?" • YES: proceed. • NO, or does not know: DO NOT SUPPLY. Refer to the GP.
2. Symptoms on exertion or during sex	• Chest pain, breathlessness or palpitations on exertion, or during previous sexual activity. • Any YES: DO NOT SUPPLY. Refer.
3. Blood pressure, measured today	• Below 90/50: exclude. • Above 170/100: exclude. • Record the reading. Do not accept a figure from memory.
4. Recent cardiac events	• Stroke or myocardial infarction in the last 6 months: exclude. • Unstable angina, angina during sex, severe heart failure or uncontrolled arrhythmia: exclude.
5. The opportunity	• Erectile dysfunction commonly precedes a cardiovascular event by 2 to 5 years, and is often the first sign of undiagnosed diabetes. • Where the patient has had no recent check, recommend one and record that you did, whether or not you supply.

Appendix 2: Every nitric oxide donor, by name

Ask about all of these. The list is the point: version 001 named only some of them, and named them in only one of the two arms.

Prescribed, obvious	• Glyceryl trinitrate: tablets, spray, patch, ointment, infusion. • Isosorbide mononitrate. • Isosorbide dinitrate.
Prescribed, easily missed	• NICORANDIL. An angina medicine. No "nitrate" in the name. It is a nitric oxide donor and it was in neither arm of version 001. • Sodium nitroprusside. • Riociguat and other soluble guanylate cyclase stimulators.
Not prescribed, will not be on any list	• AMYL NITRITE, butyl nitrite, isobutyl nitrite, alkyl nitrites. Sold as "poppers", "room odouriser" or "leather cleaner". • Ask directly, without judgement, and record the answer. He will not volunteer it.

Appendix 3: Key references

- NICE Clinical Knowledge Summary: Erectile dysfunction.
- Summary of Product Characteristics for sildenafil and for tadalafil, current versions.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

Version	v003
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Supersedes	Version 002, 8 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v003
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Full clinical review and reissue. Re-read before working to it. • One nitrate and nitric oxide donor exclusion now applies to BOTH arms, naming glyceryl trinitrate in all its forms, isosorbide mono- and dinitrate, nicorandil, amyl nitrite and the other alkyl nitrites, sodium nitroprusside and riociguat. The tadalafil arm previously said only "organic nitrates in any form", so amyl nitrite was invisible in it. Nicorandil appeared in neither arm. • A required, recorded direct question about poppers, with suggested wording, because the patient will not volunteer it and it does not appear on any medication list. • The cardiovascular fitness criterion has content: a functional question with a stated threshold, symptoms on exertion or during sex, a blood pressure measured on the day, and a requirement to record all of it. Appendix 1. • Alpha-blocker starting dose restored to the tadalafil arm: 10mg on demand, 2.5mg once daily. The sildenafil arm keeps its 25mg rule. • Over-65 dose reduction added to the tadalafil arm, which had none. Sildenafil 25mg, tadalafil 10mg on demand or 2.5mg once daily. • New exclusions in both arms: previous priapism on a PDE5 inhibitor; concurrent use of another PDE5 inhibitor including one obtained online; and erectile dysfunction of sudden onset after trauma, surgery or a new medicine, or with penile pain or deformity, which needs a diagnosis rather than a tablet. • Counselling now states that the tadalafil nitrate warning applies for up to two days after a dose because of its 17.5 hour half-life, and that a patient with chest pain after either medicine must tell the paramedics what he has taken rather than use a GTN spray. • Records must carry the nitrate and poppers answers, the cardiovascular fitness answer and today's blood pressure, alpha-blocker status, and the reason for any reduced starting dose. • Broken NICE reference corrected from mg2 to mpg2.

Previous versions

001	Development and issue of new PGD. Sildenafil and tadalafil arms, adult males 18 and over.
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Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name
 Address
 Postcode:
 ODS code, if applicable

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

[illegible]

Signed on behalf of Get Real Health

Version v003, 9 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		9 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.